A short team session: Looking after each other



For supervisors and team leaders, with known staff support and referral routes.

Before the session, read the facilitator pages from page 3.

Immediate danger? Stop the session, stay with the person and get urgent help now. Do not leave them alone. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.



Staff poster A3



Leaflet for helpers

Your materials

Staff poster · a shared focus

For the team: open the discussion, invite observations and return to the main message. Hang it where the team takes breaks; do not read every line aloud.

Leaflet for helpers · detail to keep

For a closer look and to keep. Open only the relevant section, for example the signs of stress or the ideas for every day.

Use one or both, in any order. Focus on what the team needs, not on finishing both materials.

Before the session

- ☐ Hold it at the start or end of a shift, not during a crisis. Keep it short, and make sure everyone on the team can attend, including volunteers.
- ☐ Fill in and check the staff support contacts on page 2. Hang the poster where the team can see it; bring enough leaflets.
- ☐ Make it voluntary: nobody has to share personal experiences. Plan time afterwards for anyone who wants to talk privately.

The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

Aim: the team talks openly about stress, agrees one practical change in how it works, and knows where to find support.

1 Welcome and explain the purpose

2 min · either material

DO Thank the team for their work. Explain the purpose. Joining is voluntary, and nobody has to share personal experiences.

SAY *"Stress is common in this work. Looking after ourselves is part of doing the work well."*

2 Look at the picture together

3 min · poster if useful

DO Point to the team taking a break together: tea, food, a chat, a message from home. With the leaflet only, use its cover.

ASK *"What do you see? What helps the people in the picture?"*

3 What helps

5 min · move between materials as needed

DO Go through the five things that help on the poster. Agree as a team on one practical change, for example buddy pairs, a fixed break time, or rotating the hardest tasks.

ASK *"What is one thing we can change as a team this week?"*

4 Support

3 min · relevant help section

DO Say where staff can find support and that asking is good practice. Explain how to reach you privately; use page 2 if concern arises.

SAY *"If stress gets worse or does not go away, talk to me or to staff support. You do not have to wait."*

5 Close

2 min · offer the leaflet to keep

DO Give out the leaflet and show *"Every day"* and *"Where to find support"*. Thank people again.

Safety, support and supervisor boundaries

Get urgent help now if a colleague, or anyone else:

SIGNS

- Talks about suicide, or about harming themselves or someone else
- Cannot keep themselves safe, or cannot care for a child in their care
- Seems very confused, or not aware of where they are

WHAT TO DO

1. Stay with the person. Do not leave them alone.
2. Get urgent help now: a health worker, responder or the emergency service in this area.
3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

After a distressing event in the team

- Offer psychological first aid to colleagues: stay close, listen, accept feelings, meet practical needs.
- Let people decide whether to talk. Do not hold compulsory sessions where people must retell what happened.
- Check in again over the following weeks.

PFA: Look, Listen and Link

Look: safety, urgent needs, people who are very distressed
Listen: ask about needs and concerns; help them feel calm
Link: basic needs, services, information, loved ones

Talk privately and link to further help if

- Stress keeps someone from doing their daily work or caring for themselves
- Someone uses more alcohol or drugs to cope
- Someone seems very withdrawn, or talks about not wanting to live; if so, use the urgent box above

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

Do

- ✓ Respect confidentiality
- ✓ Treat a request for support as good practice, never as weakness
- ✓ Include every team member, including volunteers and non-health staff

Do not

- ✗ Single anyone out in front of the team
- ✗ Share what someone told you without their agreement, except to keep someone safe
- ✗ Wait for people to ask before you check in

Support contacts

fill in and check before the session

Staff support _____

Health worker _____

Urgent help _____



Sources: IASC, Basic Psychosocial Skills: A Guide for COVID-19 Responders (2020); IFRC Psychosocial Centre, Caring for Volunteers; WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Guidelines for the management of conditions specifically related to stress (2013). Adapted from IASC, Basic Psychosocial Skills: A Guide for COVID-19 Responders (2020), CC BY-NC-SA 3.0 IGO, and IFRC Psychosocial Centre, Caring for Volunteers. This adaptation was not created by the IASC or IFRC, which are not responsible for its content. Licence of this adaptation: CC BY-NC-SA 3.0 IGO.

What this material is about

The poster and leaflet use everyday words, so that helpers can talk about stress without clinical labels. This page gives the meaning behind those words and what supervisors can do; page 4 holds the terms you may hear, for reference only.

Word key

ON THE POSTER OR LEAFLET	WHAT WE MEAN	HOW TO HANDLE IT IN THE SESSION
"Stress is common among helpers"	Occupational stress, including burnout and stress from hearing about or seeing others' suffering (secondary or vicarious traumatic stress)	Use *"stress"*; these terms may be used with supervisors; do not diagnose a colleague
"Ask for support: it is good practice"	Staff care and supportive supervision are part of a safe response, not a sign of weakness	Treat requests as good practice; keep them confidential
"Look out for each other"	Peer support and buddy systems	Agree practical arrangements; no compulsory sessions where people must retell what happened

Stress in helpers, and when to look further

Common stress

Tiredness, poor sleep, irritability or wanting to be alone are common in disaster response work. The person under stress is often the last to notice; colleagues can help each other see it.

Early support, at any time

If stress gets worse, does not go away, or makes work and daily life hard, talk privately and link the person to staff support or a health worker. Asking early is good practice.

Get help now

If a colleague talks about not wanting to live, or there is any danger, follow page 2 at once.

What supervisors can do

- Keep working hours reasonable and make sure people take breaks and days off.
- Check in with each person regularly, briefly and privately.
- Rotate people off the hardest tasks.
- Show appreciation.
- Make roles and expectations clear.
- Set up buddy pairs; pair new team members with experienced ones.
- Share information on confidential support.
- Model healthy habits yourself.

Keep these apart

- **Stress** is a common reaction to demanding work, not an illness.
- **Burnout** is an occupational phenomenon, not a medical condition.
- **A diagnosis** describes a lasting pattern that impairs daily life, assessed by a trained health worker.

Your limits

- You do not diagnose a colleague.
- You keep what people tell you confidential, except to keep someone safe.
- You link to staff support or a health worker; you do not act as their counsellor.

Your own support

Supervisors need support too: talk with your own manager or a peer.

Take your breaks and days off where the team can see it; that makes it easier for others to do the same.



Sources: IASC, Basic Psychosocial Skills: A Guide for COVID-19 Responders (2020), Module 1 and Annex A; IFRC Psychosocial Centre, Caring for Volunteers; WHO, Psychological First Aid: Guide for Field Workers (2011).

Terms you may hear

For reference only. Do not use these terms to label a colleague, and do not diagnose. They help you understand what a health worker or staff support may say.

TERM OR SITUATION	WHAT THE FACILITATOR NEEDS TO KNOW	SOURCE
Stress in helpers	Common and expected in disaster response work.	IASC, Basic Psychosocial Skills (2020), Module 1
Burnout	ICD-11 describes burn-out as a syndrome resulting from chronic workplace stress that has not been successfully managed. It is an occupational phenomenon, not a medical condition, and refers only to the work context.	WHO, ICD-11, QD85; WHO statement (2019)
Secondary traumatic stress and vicarious trauma	Concepts describing stress from exposure to other people's suffering and trauma stories; not diagnoses.	Figley (1995); McCann and Pearlman (1990)
After a distressing event in the team	Offer psychological first aid and choice; no compulsory sessions where people must retell what happened. Link anyone with persistent difficulties to support.	WHO, Guidelines for the management of conditions specifically related to stress (2013); IFRC Psychosocial Centre, Caring for Volunteers
***"Trauma"*, when a colleague uses the word**	Do not correct the word and do not start a discussion about trauma. Ask what the person means: *"When you say trauma, what do you mean? What do you notice in yourself?"* Helpers often use the word for a specific change, for example poor sleep, or a strong reaction to a sound or place linked to a very difficult day. Such reactions are common after a frightening or potentially traumatic experience. Respond to what the person describes, and use the early-support route if it continues, gets worse or makes work and daily life hard.	Facilitation practice for this series; listening as in WHO, Psychological First Aid (2011)

Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

Words for you to say or ask · What the person may say · Guidance for you (plain text)

WHEN A COLLEAGUE SAYS "TRAUMA"

"Since that very difficult day, I think I have trauma."

Poster: ask for support, it is good practice.

1 Ask what they mean

ASK *"When you say trauma, what do you mean? What do you notice in yourself?"*

THEY SAY *"I do not sleep well. When I hear a siren, my heart races."*

2 Name it as common

SAY *"Strong reactions after a very difficult day are common among helpers. Telling me is good practice."*

3 Agree practical support

DO Offer what the person chooses, such as rest, a fixed break, or a short time away from the hardest tasks. Do not ask them to retell what happened.

4 Know when to look further

DO If it gets worse, does not go away, or makes work and daily life hard, link the person to staff support or a health worker. Sooner if they ask.

WHEN SOMEONE WILL NOT TAKE A BREAK

"I cannot rest. People need me, and others are suffering more than I am."

Leaflet: every day, take short breaks.

1 Acknowledge their commitment

SAY *"Your work matters. You cannot take care of others for long if you do not take care of yourself."*

2 Make a break possible

ASK *"What would make it possible for you to take a short break today?"*

DO Agree a fixed break time or a buddy who covers for them.

3 Lead by example

DO Take your own breaks where the team can see it.

WHEN A COLLEAGUE PULLS AWAY

"I am fine. I just need a drink after work to sleep."

Leaflet: ways of coping that can make things worse.

1 Check in privately

ASK *"How have you been sleeping and eating lately? How are things at home?"*

2 Name your concern without blaming

SAY *"I have noticed you seem tired. Drinking more to sleep can make things worse over time."*

3 Link, and keep it confidential

DO Offer staff support or a health worker, and keep the conversation confidential. If they talk about not wanting to live, use page 2 at once.